FOLFIRINOX followed by chemoradiotherapy · rectal cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Capecitabine with radiotherapy: separate five-week phase

Active medicines: Capecitabine + Radiotherapy

Emetogenic risk: Oral minimal-to-low; radiation risk assessed separately

Use oral-systemic anticancer therapy (SACT) antiemetics as required under local policy. The radiation team assesses radiation-related emesis separately. The induction aprepitant, dexamethasone, olanzapine and atropine schedule does not continue automatically into this phase.

Additional phase-specific support

- Confirm the capecitabine prescription and radiation-day calendar: planned Days 1–5 each week for five weeks; no catch-up doses without instructions.
- Capecitabine is taken within 30 minutes after food. Use the written missed-dose, vomiting and diarrhoea plan.
- Confirm weekly blood tests and bowel, skin, hydration and radiation review. Stop capecitabine and contact the team for significant toxicity as instructed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00691 v4 · NCCP NCIS V8 oral SACT and radiation-specific guidance

Sources checked 2026-11-04 · SACTCheck v0.84.33 · Regimen-level drafts; prescriber and oncology-pharmacy verification required